

Pre-Lecture Analysis Sheet

Week 7 · Diagnosis by Inquiry — 问诊 (Wèn Zhén)

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How to use this sheet:

Before lecture → complete Sections A–D and the pre-lecture case questions. Rate your confidence honestly.

During lecture → annotate directly — add answers, corrections, and new insights in the lined spaces.

After lecture → complete Section F, update confidence ratings, and finish the clinical case.

A · Learning Targets — I Can Statements

Rate your confidence **before** lecture (Pre) and **after** lecture (Post) by circling a number 1–5 in each column. 1 = can’t do it yet · 5 = fully confident

I Can Statement	Pre-Lecture Confidence	Post-Lecture Confidence
Explain the difference between general and specific questions in TCM inquiry, and state the guiding principle for why each question is asked.	PRE ① ② ③ ④ ⑤	POST ① ② ③ ④ ⑤
List the 16 Questions of Inquiry taught in lecture and compare them to Zhang Jing-Yue’s 10 Traditional Questions.	PRE ① ② ③ ④ ⑤	POST ① ② ③ ④ ⑤
Differentiate Full vs. Empty pain characteristics (pressure, food, cold/heat, movement) using FCM Table 24.1.	PRE ① ② ③ ④ ⑤	POST ① ② ③ ④ ⑤
Interpret specific pain qualities (distending, boring/fixed, cramping, burning) and match them to their underlying pathology per Suwen 39.	PRE ① ② ③ ④ ⑤	POST ① ② ③ ④ ⑤
Correlate inquiry findings (thirst, stools, urine, sweating, sleep) with the pattern of disharmony they indicate.	PRE ① ② ③ ④ ⑤	POST ① ② ③ ④ ⑤
Apply headache location, onset, and quality to identify channel involvement and the underlying pattern.	PRE ① ② ③ ④ ⑤	POST ① ② ③ ④ ⑤

B · Activate — Connect to What You Know

B1 · From Week 6 to Today

Week 6 covered Diagnosis by **Inspection/Observation** (望诊) — reading complexion, eyes, tongue, and body language. Today’s **Inquiry** (问诊) is where you ask the questions that confirm or exclude what Observation suggested.

What do you already know about the “Ten Questions”? Write whatever comes to mind — the traditional list, why order matters, anything:

B2 · Connect the Dots

How might each of these ideas relate to today’s material? Complete the connections:

Chronic tiredness + desire to lie down → most likely maps to which organ pattern? 	Pain relieved by pressure → Full or Empty condition? Why?
Preference for cold drinks → Cold or Heat pattern? 	“Moody, irritable” emotional presentation → most likely which Liver pattern?

C · Vocabulary Pre-Load — Define Before Class

Fill in your definition/clinical note before lecture. Refine it during and after class.

Character	English	My Definition / Clinical Note
问诊 Wèn Zhén	Inquiry	4th of the Four Diagnostic Methods (望闻问切)
十问歌 Shí Wèn Gē	Song of Ten Questions	Traditional mnemonic behind today’s 16 Questions
痹症 Bì Zhèng	Bi-Syndrome	Painful obstruction syndrome (wandering/painful/fixed/heat/bone types)
五心烦热 Wú Xīn Fán Rè	Five-Palm Heat	Sweat on palms, soles & chest — Yin deficiency empty-heat

Character	English	My Definition / Clinical Note
帶下 Dài Xià	Leukorrhea	Vaginal discharge — color/consistency/smell reveal pattern
陽痿 Yáng Wǎi	Impotence	Male sexual dysfunction — several possible root patterns

Character	English	My Definition / Clinical Note
早泄 Zǎo Xiè	Premature Ejaculation	Kidney-Qi not firm, Heart-Qi or Heart-Blood deficiency
沖脈 Chōng Mài	Penetrating Vessel	Implicated in headache after orgasm & vomiting during pregnancy
魂 Hún	Ethereal Soul	Liver-housed spirit aspect involved in dreaming
神 Shén	Mind / Spirit	Anchored by sufficient Heart & Liver Blood/Yin for good sleep
異病同治 Yì Bìng Tóng Zhì	Different Disease, Same Treatment	Opening principle of the lecture (& its inverse, 同病異治)
亡陽 Wáng Yáng	Collapse of Yang	Profuse cold sweat / oily forehead sweat — emergency sign

HEADS UP
The lecture includes bonus material on **Dreams** (Suwen 17 & 80, Lingshu 43) that Prof. Schrier has stated **will NOT be tested**. No prep needed for this section — though it’s fascinating supplementary reading on Hun/Po dream theory if you’re curious.

D · Anticipatory Questions — Answer During Lecture

These questions preview exactly what the lecture will teach. Leave blank before lecture — fill in the answers as the material is covered. Come back and expand after class.

Pain 疼痛

Q1 · THE THREE CAUSES OF PAIN
According to the *Huang Di Neijing* (Suwen 39), pain arises from three main causes. What are they, and how does each interfere with the flow of Qi and Blood?
Hint: Lecture slide 8 (Pain According to Chinese Medicine).

Q2 · THE PRESSURE, FOOD & TEMPERATURE TEST ★
Complete the table showing how Empty vs. Full pain responds to pressure, food, cold, and heat:

Factor	Empty	Full
Pressure		
Food		
Cold		
Heat		

Hint: Lecture slide 18, FCM Table 24.1.

Q3 · PAIN QUALITY DIFFERENTIATION

Match each pain quality to its underlying pathology: distending/throbbing, boring & fixed/stabbing, cramping/spastic, and burning.

Hint: Lecture slide 19 (Causes of Pain).

Food, Drink & Elimination

Q4 · THIRST WITHOUT DESIRE TO DRINK

A patient says they are thirsty but have no desire to actually drink liquids. What pattern does this indicate, and why is this a good example of a “terminology problem” in TCM inquiry?

Hint: Lecture slide 2 (Problems with Terminology) & slide 13 (Thirst).

Q5 · THE STOOLS & BLOOD SEQUENCE RULE ★

How does whether blood comes before or after the stool — and whether the stool is foul-smelling or odorless — help you distinguish Damp-Heat in the Intestines from Spleen-Qi deficiency unable to hold Blood?

Hint: Lecture slides 30–31 (Stools & Diarrhea, continued).

Sleep & Sweating

Q6 · THE SPIRIT & SLEEP CONNECTION

Which two organs’ Blood/Yin must be sufficient to “anchor” the Mind/Shen for good sleep? Contrast an insomnia pattern from Heart & Spleen Blood deficiency with one from Kidney & Heart Yin deficiency with Empty-Heat.

Hint: Lecture slides 20–21 (Sleeping).

Q7 · SWEATING LOCATION RULE ★

What does sweating confined only to the head indicate? What does sweat confined to the palms, soles, and chest (“five-palm heat”) indicate?

Hint: Lecture slide 26 (Area of Sweating).

Head & Face

Q8 · HEADACHE LOCATION BY CHANNEL ★

Fill in the table below with the channel(s) involved and the possible pattern(s) that can cause pain at each location:

Location	Channel(s)	Possible Cause(s)
Nape of neck		
Forehead		
Temples & sides		
Vertex		
Whole head		

Hint: Lecture slide 14 (Head — Headache Location).

Q9 · ONSET & TIME OF DAY

How does headache onset (recent/sudden vs. gradual) and time of day (daytime vs. evening vs. night-time) help distinguish an exterior invasion from Qi/Yang deficiency, Blood/Yin deficiency, and Blood stasis?

Hint: Lecture slide 14 (Head — Headache Onset and Time of Day).

Q10 · FULL VS. EMPTY EMOTIONAL PRESENTATION

Contrast an Empty-condition presentation of depression or anxiety with a Full-condition presentation. Name at least two distinct organ patterns for each.

Hint: Lecture slides 30–31 (Depression & Anxiety — Empty/Full Conditions).

E · Clinical Case — Apply It Before & After

Case Vignette: Ms. Delgado, 52 years old

Ms. Delgado is a 52-year-old female with a 6-month history of dull, gnawing epigastric pain that is **relieved by eating and by pressure**, and worsens when she is tired or skips a meal. She describes **chronic tiredness**, a desire to lie down after meals, and **poor appetite**. Her stools are **loose and occur first thing in the morning**. She feels **cold in her hands and feet** and prefers **warm drinks taken in small sips**, though she isn’t especially thirsty. Her sleep is unremarkable. On examination: tongue is **pale and slightly swollen with tooth marks**. Pulse is **deep and weak** (沉虛 Chén Xū).

- Epigastric pain — relieved by eating/pressure
- Chronic tiredness
- Poor appetite
- Loose stools — early AM
- Cold hands/feet
- Warm drinks, small sips
- Tongue: pale, swollen, tooth-marked
- Pulse: 沉虛 Chén Xū

Pre-Lecture Questions — Answer Based on Your Current Knowledge

Before the lecture, do your best with these. Guessing is fine — it primes your brain to receive the lecture material.

PRE · Q1

Is her epigastric pain a Full or Empty presentation? Which two inquiry findings support your answer?

PRE · Q2

Which organ pattern does chronic tiredness + poor appetite + loose stools + desire to lie down most likely point to?

PRE · Q3

Circle: COLD — HEAT. List the inquiry evidence (drink preference, limb temperature) supporting your choice.

Post-Lecture Questions — Complete After Class

POST · Q1

State Ms. Delgado’s complete pattern using proper TCM terminology (e.g., “Spleen-Qi deficiency” or more specific).

POST · Q2

What additional questions from the 16 Questions framework would help confirm or exclude Kidney-Yang deficiency as a contributing factor?

POST · Q3

If untreated and Spleen-Qi deficiency progresses toward Spleen-Yang deficiency, what additional inquiry findings might emerge?

F · After-Lecture Synthesis & Self-Monitoring

Complete this section within 24 hours of lecture for maximum retention.

I Can Now Confidently...

Still Unclear — I Need to Review...

Connected to Other Learning...

Procedures for Inquiry — Sequence Map

Complete the sequence using only what you can recall. Fill in the blank steps:

1

Observe the patient — complexion, movement, eyes — on entry & throughout

2

3

4

5

Tongue & Pulse — confirm/exclude patterns, add info not evident from consultation

Homework Tracker

Assigned Homework (from lecture):

☐ **Task 1:** Complete the three “Test Your Knowledge” fill-in-the-blank slides from lecture (Thirst; Pain in Hypochondrium; Joint Pain) using your own notes — check against the Cram Sheet once posted.

☐ **Task 2:** Conduct a full 16 Questions intake on a classmate, friend, or family member and record the Eight Principles + Zang-Fu pattern that emerges.

Notes on finding participants / logistics: